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Rotator Cuff Tendinopathy: The New 2025 Clinical Practice Guideline



Rotator cuff-related shoulder pain is one of the most common musculoskeletal complaints presenting in primary care and physiotherapy clinics. Yet, overmedicalization, inconsistent assessment strategies, and reliance on imaging remain widespread.

The 2025 Clinical Practice Guideline by Desmeules et al., published in the Journal of Orthopaedic & Sports Physical Therapy, delivers a comprehensive and evidence-based overview for the diagnosis,
non-surgical management, and rehabilitation of adults with rotator cuff tendinopathy (RCT). This article summarizes the key updates and takeaways from the 2025

guideline, helping clinicians apply them with confidence and clarity in everyday practice.

> **THE 2025 CLINICAL PRACTICE GUIDELINE DELIVERS A COMPREHENSIVE AND EVIDENCE-BASED OVERVIEW FOR THE DIAGNOSIS, NON-SURGICAL MANAGEMENT, AND REHABILITATION OF ADULTS WITH ROTATOR CUFF TENDINOPATHY.**

Why This Guideline Matters

Developed by an international team of experts across physiotherapy, orthopaedics, and physical medicine, the clinical practice guideline is grounded in systematic reviews, patient involvement, and rigorous methodological standards. Backed by the Quebec Rehabilitation Network (REPAR), Quebec Pain Research Network (QPRN), and the Academy of Orthopaedic Physical Therapy (AOPT) of the American Physical Therapy Association (APTA), it consolidates the best available evidence for clinicians worldwide. With eight patient representatives from three countries involved in the revision process, this guideline also reflects the lived experience and preferences of those receiving care.

Evidence Grades Explained

Each recommendation in the clinical practice guideline is assigned a grade reflecting the strength and certainty of the supporting evidence:

- **Grade A** – Strong evidence: Supported by multiple high-quality randomized controlled trials (RCTs) or systematic reviews.
- **Grade B** – Moderate evidence: Supported by at least one high-quality RCT or several lower- quality RCTs or cohort studies.
- **Grade C** – Weak evidence: Based on single studies of lower quality or expert consensus where research is limited.
- **Grade D** – Conflicting evidence: Studies disagree in their findings.
- **Grade E** – Theoretical/foundational: Based on cadaveric, biomechanical, or animal research.
- **Grade F** – Expert opinion: Based on best practices and clinical consensus in the absence of empirical evidence.

Key Clinical Recommendations

1. Assessment & Diagnosis

- **Comprehensive Subjective History (Grade F)**: Clinicians must include a thorough patient history considering age, gender, hand dominance, job demands, sport activities, medication use, comorbidities, psychosocial influences, mechanism of injury, previous treatments, current symptoms (pain, range of motion [ROM] limitation, weakness), and patient goals.
- **Physical Assessment (Grade F)**: Should include inspection for deformity, muscle atrophy, and swelling; measurement of active/passive ROM and strength; and optional palpation and special tests based on clinical reasoning. Cervical spine should be screened to rule out referred pain.
- **Red Flags (Grade F)**: Clinicians must screen for serious pathologies such as infection, cancer, cardiovascular conditions, or systemic involvement. Prognostic Factors (Grade B): Identification of personal, clinical, and work-related factors affecting prognosis is recommended to guide individualized care plans.
- **Special Tests (Grade B)**: Painful arc test can help confirm diagnosis, while the Hawkins-Kennedy test can assist in ruling it out.
- **Objective Measures (Grades A)**: ROM should be measured using a goniometer,

inclinometer, or validated smartphone application. Scapular ROM is unreliable and not recommended. Strength testing should be performed using a handheld dynamometer.

- **Patient-Reported Outcome Measures (Grade A)**: Use validated, reliable, and responsive tools such as the Shoulder Pain and Disability Index (SPADI) or the Disabilities of the Arm, Shoulder and Hand (DASH) questionnaire to track pain and disability.
- **Imaging (Grades F)**: Should not be used initially. Diagnostic ultrasound is preferred if imaging is needed after 12 weeks of failed conservative care. Magnetic Resonance Imaging (MRI) is not routinely recommended. Discuss pros/cons of imaging openly with patients.
- **Referral (Grade F)**: Patients with persistent and severe symptoms after 12 weeks of care should be referred to a musculoskeletal specialist (e.g., sports physician, physiatrist, or orthopedic surgeon).

2. Pharmacological Treatment

- **Acetaminophen (Grade C)**: May be recommended for short-term pain relief. Non-Steroidal
- **Anti-Inflammatory Drugs (NSAIDs) (Grade B)**: Effective for short-term pain management.
- **Opioids (Grades F/C)**: Should not be used as first-line treatment. May be considered short-term in severe cases where other options are ineffective or contraindicated. Requires regular risk reassessment.
- **Corticosteroid Injections (Grades B/C)**: May be used for short-term pain relief but not as a first-line intervention. Ultrasound guidance is recommended if injections are given.
- **Calcific Lavage (Grade B)**: Recommended for calcific tendinopathy that is unresponsive to initial treatments.
- **Platelet-Rich Plasma (PRP) & Hyaluronic Acid (Grades D/F)**: May be considered

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3. Rehabilitation Principles

- **Education (Grade C)**: Clinicians should educate patients on their condition, activity modification, pain neuroscience, prognosis, and self-management strategies. Tailor communication to the level of literacy and psychosocial context.
- **Exercise Therapy (Grade A)**: Core intervention. Should include progressive resistance training and motor control exercises. Individualize based on pain tolerance and patient goals.
- **Manual Therapy (Grade B)**: May reduce pain short-term when used with exercise. Techniques include soft tissue work and joint mobilizations/manipulations.
- **Taping (Grade D)**: Can be used adjunctively to reduce pain short-term.
- **Acupuncture (Grade C)**: May offer additional short-term benefits when combined with active rehab.
- **Shockwave Therapy (Grade C)**: Useful in calcific tendinopathy. Not recommended in non-calcific RCT.
- **Laser Therapy (Grade C)**: May reduce pain in calcific tendinopathy.
- **Therapeutic Ultrasound (Grades C/B)**: Not recommended for either calcific or non-calcific RCT due to lack of benefit.
- **Ergonomic Modifications (Grade C)**: May help reduce occupational stress on the

shoulder.

4. Return to Sport

- **Load Tolerance (Grade F)**: Return-to-sport plans should be based on athlete's ability to tolerate shoulder and rotator cuff loading.
- **Outcome Measures (Grade F)**: Use validated tools to assess pain, disability, readiness to return, and functional performance. Examples include sport-specific performance tests and return-to-sport checklists.

Summary

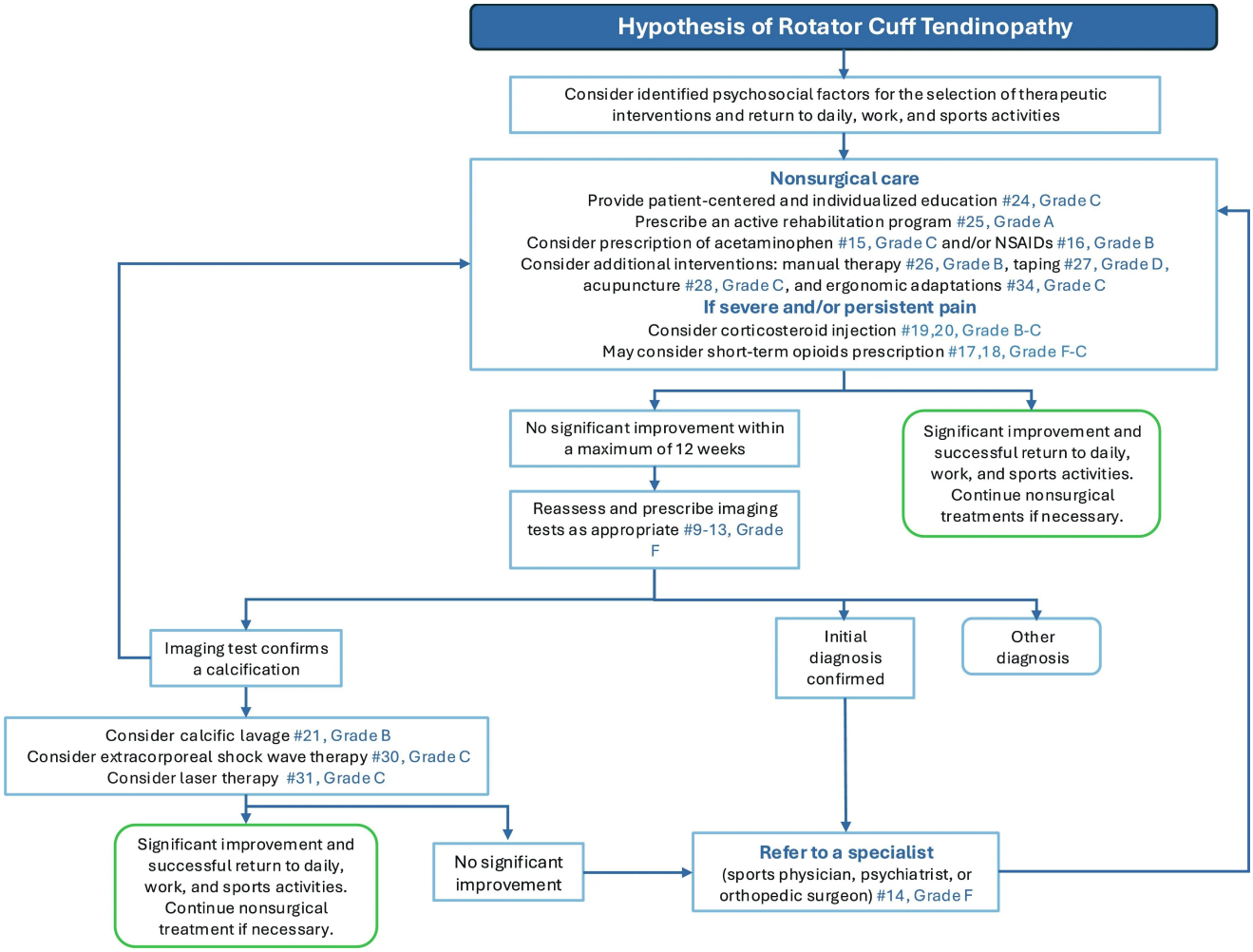
The 2025 Clinical Practice Guideline for rotator cuff tendinopathy delivers a clear and comprehensive framework for conservative shoulder care. It prioritizes individualized assessment based on patient history and clinical examination, discourages early imaging, and supports objective measurement tools for range of motion and strength. Pharmacological interventions are secondary to active rehabilitation, with exercise therapy positioned as the cornerstone of recovery. Adjunct therapies like manual therapy, taping, and acupuncture may be considered selectively. The guideline also highlights the importance of patient-centered education and gradual, load-based return-to-sport programming. Overall, it encourages clinicians to rely less on diagnostic imaging and passive modalities, and more on active, evidence-informed strategies that empower patients.

Changes from the 2022 guideline:

- Integration of systematic reviews up to October 2023.
- New recommendations specifically addressing return to sport.
- Clearer grading of evidence using GRADE-adapted methodology.
- Enhanced patient-centered language and prioritization of shared decision-making.

The Rotator Cuff Management Decision Tree

The authors provided an additional decision tree that guides clinicians in their patients process of rehabilitation from rotator cuff tendinopathy.



Management of Rotator Cuff Tendinopathy decision tree. The symbol # represents the associated recommendation number (outlined in the paper). Grade letters indicate that the guidelines are based on (A) strong evidence, (B) moderate evidence, (C) weak evidence, (D) conflicting evidence, (E) theoretical/foundational evidence, or (F) expert opinion.

Limitations & Conclusion

While this guideline is extensive, some areas, such as return to sport timelines and PRP effectiveness, still lack high-quality RCTs. Further research is needed to individualize treatment and identify prognostic subgroups. The 2025 Rotator Cuff Tendinopathy CPG offers an evidence-based, patient-centered approach that can reduce overtreatment and improve clinical outcomes. By aligning practice with these recommendations, clinicians can more confidently assess, educate, and rehabilitate adults with RCT.

Reference

Desmeules, F., Roy, J-S., Lafrance, S., et al. (2025). Rotator Cuff Tendinopathy Diagnosis, Non-Surgical Medical Care and Rehabilitation: A Clinical Practice Guideline. Journal of Orthopaedic & Sports Physical Therapy. https://doi.org/10.2519/jospt.2025.13182